

LRT-4051 Phase 1 Outsourcing Options

Sourcing options for the first-in-human study of LRT-4051, for discussion with the Chief Development Officer and the CFO on August 20, 2026. No vendor is being selected today.

Decision requested

Agree which sourcing models go into the request for proposal planned for November and December 2026, and agree the evaluation criteria. No vendor choice and no budget commitment are requested today; no first-in-human sourcing decision has been made.

Why decide the model now

LRT-4051 is an investigational oral small molecule for diabetic kidney disease, nominated at G2 in September 2025. The FY2026 plan has the IND submitted on April 30, 2027. GLP repeat-dose in-life phases finished in late July, about two weeks after plan. The first-in-human study will be outsourced: we have no clinical pharmacology unit and no in-house data management.

Option A – full-service CRO

One partner runs the unit relationship, data management, biostatistics, safety reporting support and medical writing. This is closest to how CANOPY-1 works: Halvorsen performs site activation, monitoring, data management, the eTMF and pass-through payments, while we keep the protocol, medical monitoring oversight, safety reporting decisions and regulatory submissions. Least coordination for our team; least control over unit choice.

Option B – Phase 1 unit plus functional vendors

Contract a clinical pharmacology unit directly and add separate vendors for data management and biostatistics. Merrow Clinical Pharmacology Unit is completing a feasibility assessment. Gives us the most say over the unit, dosing schedule and data standards; needs more sponsor coordination and three contracts instead of one.

Option C – extend an existing CRO relationship

Add the study to an existing master agreement with a qualified CRO, which would place the unit as a subcontractor. Fastest contracting route and a known team; the unit would be one step removed, so we would oversee a subcontracted activity.

Evaluation criteria

Unit availability for a start after the IND is in effect.

Sponsor oversight load on Clinical Operations in 2027, while CANOPY-1 runs to database lock.

Fit with our data standards and transfer checks.

Qualification: tier 1 partners need QA approval and a quality agreement before a work order.

Cost and contracting time; approval level depends on total value, including pass-through.

Preliminary assessment

Options B and C look stronger than A on control and speed, but the evidence is thin. Cost is compared qualitatively here; the pricing benchmark mentioned in July was not circulated with this deck. No option is recommended yet.

Information still needed

Merrow feasibility assessment and QA qualification outcome.

Confirmed IND timing once GLP report dates are firm.

Clinical Operations capacity plan for 2027.

Our GLP bioanalysis partners' ability to support clinical PK: Alder Bioanalytical and Kestrel DMPK Services are already on the IND-enabling program.

Timeline to request for proposal

August to October – feasibility, requirements and short list of models.

November to December – request for proposal to shortlisted partners.

First quarter 2027 – selection, qualification close-out and contracting at the approval level the value requires.